

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<reasoning process here> The boy presents with symptoms of inattention, impulsivity, hyperactivity, poor concentration, and disruptive behavior, which are classic signs of attention-deficit/hyperactivity disorder (ADHD). His history of aggression, difficulty focusing, and behavioral issues in preschool settings supports this diagnosis. Although he has a family history of major depressive disorder and Graves' disease, these do not directly explain the current symptoms. The fact that he meets all developmental milestones and has no other medical abnormalities makes ADHD a likely diagnosis.

The next step in treatment for a child with ADHD is typically behavioral therapy, especially in the early stages or when medication is not immediately indicated. While stimulant medications like methylphenidate are common first-line treatments for ADHD, and SSRIs like fluoxetine may be used in comorbid anxiety or depression, the most appropriate first-line treatment in a child with primary ADHD, especially in a preschool setting, is behavioral intervention.

Methimazole (B) is used for hyperthyroidism and is not relevant here. Hearing aids (D) are indicated for hearing loss, which is not suggested by the history. Fluoxetine (C) may be used for comorbid anxiety or depression, but there is no mention of such symptoms.

Therefore, the most appropriate next step is behavior therapy, which is recommended as a first-line intervention for ADHD in children, especially when the child is young and the family is seeking non-pharmacological approaches.

<answer> A. Behavior therapy </answer>